SOP No:	Standard Operating Procedure	Revision: 01
PV-Literature Screening		
Issue date:	Effective date:	Next revision:

Standard Operating Procedure

Of

PV-Literature Screening

	Name	Title	Signature	Date
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Reviewed by		QA specialist		
Approved by		CEO		

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1. Purpose:

1.1. To describe how the medical and scientific literature is screened for the active ingredients of Sato Pharma's products, so that:

1.1.1. Every report of a suspected adverse reaction published in the literature is found, assessed and, when it is a valid individual case, reported to the Egyptian Drug Authority on time.

1.1.2. Every literature reference is screened at least once a week, and the result of each screening, including a negative result, is recorded.

1.1.3. The screening is documented for the PSUR, the pharmacovigilance system master file and inspection.

2. Scope:

2.1. Applies to the screening of the global and local literature for every active ingredient on the PHAROS watchlist, by the pharmacovigilance department, and to the handling of the individual cases it finds.

2.2. The screening of regulatory authorities' safety publications in PHAROS is outside this SOP, except where it shares the weekly report.

3. Abbreviations:

3.1. SOP: Standard Operating Procedure.
3.2. PV: Pharmacovigilance.
3.3. QPPV: Qualified Person for Pharmacovigilance.
3.4. QA: Quality Assurance.
3.5. CAPA: Corrective Action Preventive Action.
3.6. ICSR: Individual Case Safety Report.
3.7. EDA: Egyptian Drug Authority.
3.8. EMA: European Medicines Agency.
3.9. PMC: PubMed Central.
3.10. PSUR: Periodic Safety Update Report.
3.11. E2B(R3): the international format for the electronic transmission of ICSRs.
3.12. GVP: Good Pharmacovigilance Practice.

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4. Definitions:

4.1. **PHAROS:** Sato Pharma's pharmacovigilance screening system. It searches the literature databases, keeps the search log and the hand checks, flags possible case reports and holds the individual cases.

4.2. **Literature reference:** a database or journal in which the literature is screened. **Global** references are the international databases (PubMed, PubMed Central, Europe PMC, the Cochrane Database of Systematic Reviews, Medscape). **Local** references are the Egyptian medical journals the EDA asks for (Al-Azhar Medical Journal, Ain Shams Medical Journal).

4.3. **Searched by the app:** a reference PHAROS searches itself for each ingredient's names. **Checked by hand:** a reference PHAROS cannot search, which a member of the PV department searches and records every week.

4.4. **Nothing found:** the recorded negative result of a reference for an ingredient. PHAROS writes it only when the ingredient's own name was searched in the period and no read of that database failed, or when a hand check found nothing.

4.5. **Possible case report:** a paper flagged by PHAROS because PubMed lists it as a case report or its title says “case report”, “case series” or “case of”, and its title or abstract names the ingredient.

4.6. **Valid ICSR:** a case with the four minimum criteria: an identifiable patient, an identifiable reporter, a suspect product and a suspected adverse reaction.

4.7. **Day zero:** for a literature case, the Cairo day PHAROS first found the paper, never its publication date.

4.8. **Deviation:** as defined in PV-SOP-021.

5. Responsibilities:

5.1. **QPPV:**

- 5.1.1. Owns the literature screening and this SOP.
- 5.1.2. Keeps the list of literature references current in PHAROS (Sources, Literature references).
- 5.1.3. Reviews and signs the weekly report.
- 5.1.4. Assesses each possible case report, decides its validity and seriousness, and makes sure valid cases reach the EDA on time.
- 5.1.5. Notifies QA of any deviation from this SOP, as in PV-SOP-021.

5.2. **PV officer (a reviewer in PHAROS):**

- 5.2.1. Reviews each ingredient's literature every week and records a decision on every possible case report.
- 5.2.2. Performs and records the weekly hand checks.
- 5.2.3. Opens, checks and prepares the individual cases in PHAROS for the QPPV.

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5.3. **QA department:**

5.3.1. Evaluates deviations and CAPA from this SOP under PV-SOP-021, and audits it under PV-SOP-022.

6. Introduction:

6.1. Marketing authorisation holders must screen the worldwide literature for reports of suspected adverse reactions to their active ingredients, and report valid individual cases to the competent authority. Sato Pharma does this in PHAROS: the international databases are searched automatically, every search is logged and can never be changed, and the references PHAROS cannot search are checked by hand and recorded in the same place. The weekly report keeps the result of every reference, week by week, as the record of the screening.

7. Procedure:

7.1. **Automatic searches.** PHAROS searches PubMed, PubMed Central and Europe PMC four times a day for every ingredient on the watchlist, by its English names, in titles and abstracts. A drug class is searched only when it is set “In the literature search” on the ingredient. Each search is logged with the database, the words, the period and the number of records returned, 0 included. Cochrane reviews are counted from these databases by their journal.

7.2. **Weekly review of each ingredient.** Every week the PV officer opens each ingredient on the Ingredients screen for the last week and, under Literature:

7.2.1. Reads the possible case reports first, then the other papers.

7.2.2. Records a decision on each possible case report: Concerns us, with a comment, or Doesn't concern us, with the reason.

7.2.3. Reads the Literature references table: for each reference, how it was screened and what was found. A line saying a read failed, or that a reference was not searched, is not a negative result: that database is searched by hand for the ingredient.

7.3. **Weekly hand checks.** Every week the PV officer searches each reference marked By hand (Medscape, Al-Azhar Medical Journal, Ain Shams Medical Journal) for every ingredient on the watchlist, and records it on Sources, Literature references:

7.3.1. Nothing found: Checked: nothing found, then Record: nothing found.

7.3.2. Something found: Checked: found something, with what was found (the article and its link or reference), then Record the check. A possible case is then assessed as in 7.5.

7.3.3. A recorded check cannot be changed or deleted. A reference showing “not checked this week” is still to do.

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7.4. **EMA's literature monitoring.** Each ingredient's screen shows whether EMA's medical literature monitoring covers it. The QPPV takes this into account when deciding what is reported, in line with Egypt GVP.

7.5. **Assessing a possible case.** The QPPV, or the PV officer for the QPPV, reads the full paper and decides whether it describes a valid ICSR for one of our products (the four minimum criteria) and whether it is serious.

7.6. **Entering the case.** A valid or possibly valid case is opened in PHAROS:

7.6.1. From the paper: press Open a case on the possible case report. PHAROS fetches the paper and fills the case form; day zero is the day PHAROS found the paper.

7.6.2. From a document: upload it on Cases, New case, with its day zero; or forward it to the PHAROS mailbox with the word CASE in the subject.

7.7. **Checking and confirming.** The PV officer checks every field of the case form against the source, corrects it and saves; sets the seriousness (Serious, 15 days; Not serious, 90 days); and chooses the product and its strength. Confirm the case is refused while a minimum criterion is missing, the seriousness is not decided or no product is chosen. A case that is not valid is closed with Not a valid case and a comment; no case is ever deleted.

7.8. **Reporting to the EDA.** The case is sent to the EDA within 15 calendar days of day zero if serious, 90 if not, and then recorded in PHAROS with Sent on and the authority's reference. The clock shows on the Cases list until the case is sent.

7.8.1. While Sato Pharma has no MedDRA subscription, the case is sent on the CIOMS I form: press Open the CIOMS I form on the confirmed case, save it as PDF and send it. The reactions are given in the reporter's words, so no MedDRA code is needed; each reaction is marked as needing official MedDRA coding.

7.8.2. Once MedDRA is available, each reaction is coded with its MedDRA Lowest Level Term and version, and PHAROS prepares the E2B(R3) file. PHAROS never chooses a MedDRA code itself. Until a first file has passed the authority's gateway test, the E2B(R3) file is a prepared draft and is checked before it is sent.

7.8.3. A term from a free terminology (NCI Thesaurus, OAE, ICD-11, MeSH) may be recorded on a reaction. It is not MedDRA and is never sent in the E2B(R3) file.

7.9. **Follow-up information.** New information on a confirmed or sent case is entered after Reopen with a comment, and a new day zero when it is a follow-up.

7.10. **The weekly report.** Every Sunday at 05:00 Cairo time PHAROS makes the weekly report (PDF and Excel) for the week Sunday to Saturday. Section 02 lists each literature reference and how it was screened; section 04 gives, for each ingredient, its possible case reports, its references with what was found, its searches and its papers. The QPPV downloads both files, files them with their SHA-256, signs the report in PHAROS and works through section 06, To check by hand.

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7.11. **PSUR.** For a PSUR, the PSUR pack's section 11 (Literature) gives each reference over the interval, how often it was searched or checked by hand and what was found, with the possible case reports first.

7.12. **Deviations and CAPA.** Each of the following is a deviation, notified by the QPPV to QA on the deviation report (PV-FM-009) and handled under PV-SOP-021, with its root cause and CAPA on the CAPA form (PV-FM-010):

- 7.12.1. A weekly hand check not recorded for the week.
- 7.12.2. A failed or missing read not followed by a hand search.
- 7.12.3. A valid case sent to the EDA after 15 days (serious) or 90 days (not serious) from day zero.
- 7.12.4. A weekly report not reviewed and signed, or its section 06 not worked through.

7.13. **Audits.** An observation on the literature screening raised in an internal or external audit is answered with a CAPA plan under PV-SOP-022 (PV-FM-011 to PV-FM-013). The evidence is the weekly reports with their SHA-256 and signatures, the search log, the hand checks, the Cases history and the audit log in PHAROS. PHAROS keeps no deviation or CAPA record of its own.

8. Forms / Templates / Records:

8.1. Weekly report, PDF and Excel (PHAROS): sections 02 and 04; Excel sheets Literature references, Literature searches and Literature papers.

8.2. Daily report, PDF and Excel (PHAROS), made at 07:00 Cairo time for the day before.

8.3. PSUR pack, section 11 Literature; Excel sheet §11 References.

8.4. Cases in PHAROS: the source document with its SHA-256, the case form, the history of every step, and the CIOMS I form or E2B(R3) file sent.

8.5. Deviation report: PV-FM-009.

8.6. CAPA form: PV-FM-010.

8.7. Audit plan form: PV-FM-011.

8.8. Audit report form: PV-FM-012.

8.9. CAPA plan: PV-FM-013.

9. References:

9.1. Guidelines for Good Pharmacovigilance Practice in Egypt V.2.

9.2. Guideline on good pharmacovigilance practices (GVP), Module VI: Collection, management and submission of reports of suspected adverse reactions to medicinal products (EMA).

9.3. PV-SOP-021 Deviation management.

9.4. PV-SOP-022 Internal and external PV audit.

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9.5. PV-SOP-024 Quality Management System.
9.6. The PHAROS Handbook, edition 4, November 2026.

10. Process map / flowchart / Organogram:

10.1. PHAROS searches PubMed, PubMed Central and Europe PMC four times a day.
10.2. Weekly: review each ingredient's literature and decide on each possible case report.
10.3. Weekly: check by hand each reference marked By hand and record it.
10.4. Possible case: read the paper, assess validity and seriousness.
10.5. Valid case: open it on Cases, check, confirm.
10.6. Send to the EDA within 15 or 90 days of day zero; record as sent.
10.7. Sunday: weekly report signed and filed; gaps raised as deviations (PV-SOP-021).

11. Change history:

Revision no.	Issue date	Changes
01		Initial SOP

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